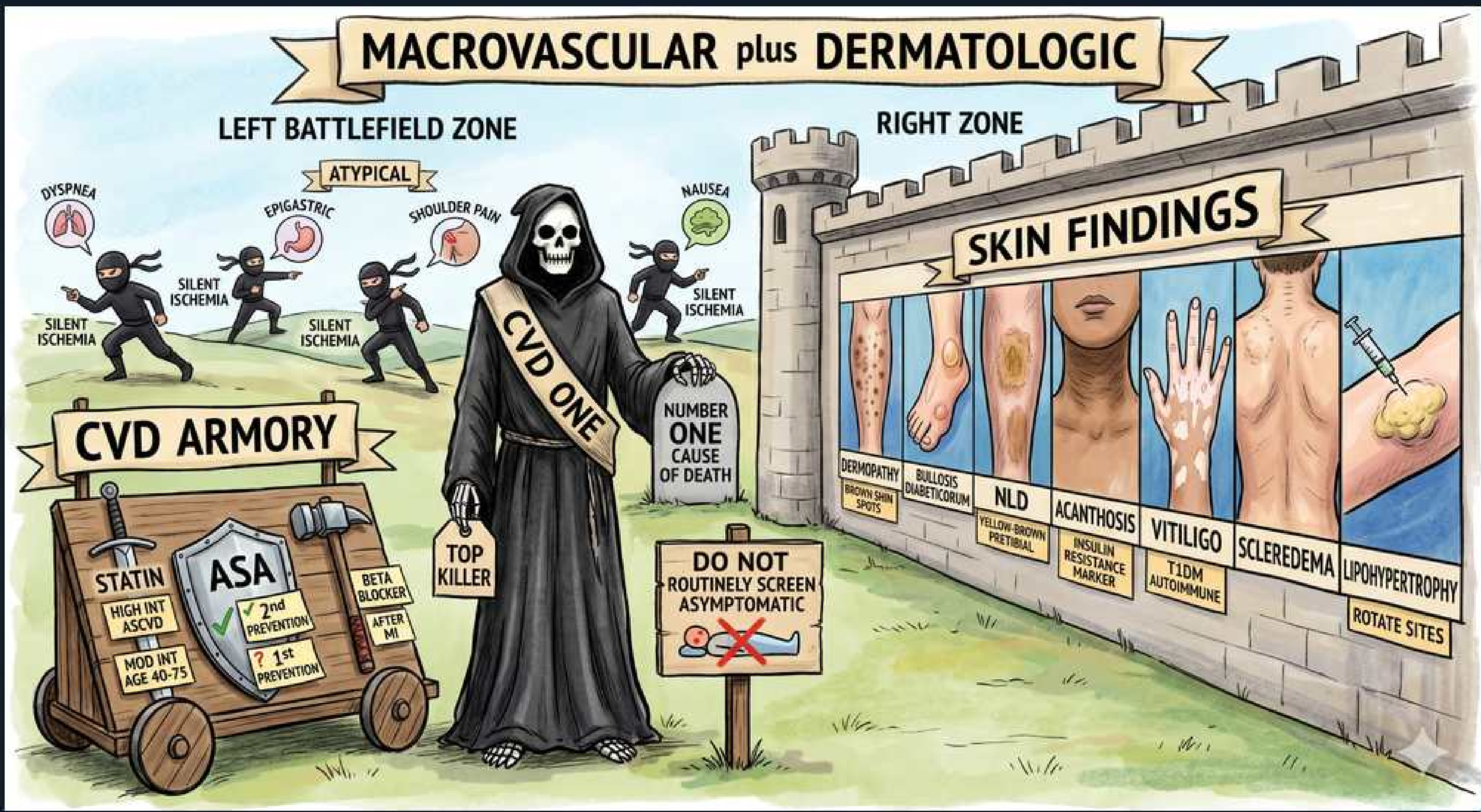
DM — Scene 4B — Diabetic Complications: The CVD Battlefield & Skin Mural (REBUILD - SPLIT)



1. Macrovascular + dermatologic banner

WHAT YOU SEE

A castle-wall banner reading MACROVASCULAR plus DERMATOLOGIC over a battlefield (left) and a tiled skin mural (right) — splitting the scene into large-vessel disease and the diabetic skin findings.

WHAT IT ENCODES

This scene covers the MACROVASCULAR complications of diabetes (CAD, stroke, PAD — large-vessel atherosclerosis) plus the characteristic DERMATOLOGIC findings. Macrovascular disease is driven by accelerated atherosclerosis from hyperglycemia, dyslipidemia, hypertension, and a prothrombotic state, and is the chief cause of diabetic mortality. Contrast with microvascular disease (retinopathy, nephropathy, neuropathy), which tracks more tightly with glycemic control. Mnemonic: 'macro kills, micro disables.'

BOARD TRAP

WRONG to assume tight glycemic control alone prevents macrovascular events — unlike microvascular disease, macrovascular risk responds most to BP, statin, and antiplatelet/risk-factor control. Discriminator: ACCORD showed intensive glucose lowering did not reduce (and may worsen) macrovascular outcomes.

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2. Silent ischemia ninjas

WHAT YOU SEE

Ninjas labeled SILENT ISCHEMIA across the left battlefield

WHAT IT ENCODES

Diabetics frequently have SILENT myocardial ischemia and infarction because cardiac autonomic neuropathy blunts the anginal afferent signal. The result is missed or delayed MI presentations and higher mortality. Maintain a low threshold to investigate diabetics with atypical or absent symptoms (e.g., new dyspnea, fatigue, CHF). Mnemonic: 'no pain still means no oxygen.'

BOARD TRAP

WRONG to exclude an MI in a diabetic because there is no chest pain — absence of chest pain does NOT exclude MI in a diabetic. Discriminator: obtain an ECG and troponin for unexplained dyspnea, nausea, or hemodynamic change rather than relying on the classic crushing chest pain that may never appear.

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3. Atypical symptoms (dyspnea, epigastric, nausea)

WHAT YOU SEE

Labels ATYPICAL: DYSPNEA, EPIGASTRIC, SHOULDER PAIN, NAUSEA

WHAT IT ENCODES

Diabetic MI commonly presents ATYPICALLY: dyspnea (anginal equivalent), epigastric pain, nausea/vomiting, diaphoresis, shoulder/jaw pain, or new heart failure — often without classic substernal chest pain, again due to autonomic neuropathy. Women, the elderly, and diabetics share this atypical pattern. Mnemonic: 'in diabetics, the MI whispers.'

BOARD TRAP

WRONG to attribute a diabetic's epigastric pain and nausea to gastritis/GERD without an ECG — these can be an anginal equivalent. Discriminator: in a diabetic with cardiovascular risk factors and atypical upper-GI or respiratory symptoms, rule out ACS (ECG + troponin) before settling on a benign GI diagnosis.

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4. CVD One reaper / top killer

WHAT YOU SEE

Grim reaper labeled CVD ONE, NUMBER ONE CAUSE OF DEATH, TOP KILLER

WHAT IT ENCODES

Cardiovascular disease (CAD, stroke, PAD) is the LEADING cause of death in diabetes, accounting for the majority of mortality; diabetes confers roughly a 2-fold increase in CV risk. This is why management centers on aggressive risk-factor control: statins, BP control (target <130/80), smoking cessation, and consideration of SGLT2 inhibitors/GLP-1 RAs that independently reduce CV events. Mnemonic: 'diabetics die of their hearts, not their sugars.'

BOARD TRAP

WRONG to think renal failure or hyperglycemic crisis is the number-one killer in diabetes — atherosclerotic CARDIOVASCULAR disease is. Discriminator: the highest-yield mortality-reducing interventions are statins and BP control, not just lowering A1c.

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5. Do not routinely screen (asymptomatic)

WHAT YOU SEE

Sign DO NOT ROUTINELY SCREEN, ASYMPTOMATIC, crossed-out heart

WHAT IT ENCODES

Do NOT routinely screen ASYMPTOMATIC diabetics for CAD with stress testing or coronary imaging — trials (e.g., DIAD) showed no improvement in outcomes because everyone already gets aggressive risk-factor treatment. Resources go to statins, BP, glucose, and lifestyle instead. Test only when symptoms, an abnormal resting ECG, or signs of vascular disease appear. Mnemonic: 'treat the risk, don't chase the asymptomatic scan.'

BOARD TRAP

Routine cardiac stress testing in an asymptomatic diabetic is the WRONG answer — it does not improve outcomes. Discriminator: pursue functional/anatomic cardiac testing only for SYMPTOMS (angina equivalent, dyspnea) or abnormal baseline ECG, not as part of routine surveillance.

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6. Statin (high intensity)

WHAT YOU SEE

A large sword in the CVD ARMORY cart labeled STATIN, HIGH INT ASCVD and MOD INT AGE 40–75 — the sharpest weapon = the statin every at-risk diabetic gets.

WHAT IT ENCODES

Statins are first-line for nearly all diabetics: HIGH-intensity (atorvastatin 40–80 mg or rosuvastatin 20–40 mg) for established ASCVD or high 10-year risk; MODERATE-intensity for ages 40–75 without ASCVD. Add ezetimibe ± a PCSK9 inhibitor if LDL remains above goal in very-high-risk patients. Mnemonic: 'every diabetic 40–75 gets a statin.'

BOARD TRAP

WRONG to base statin initiation on the LDL number alone or to withhold it because LDL is 'normal' — in diabetes, statins are given for RISK, not just to a fixed LDL target. Discriminator: a 55-year-old diabetic with LDL 95 still warrants a statin (moderate-to-high intensity) based on diabetes-driven ASCVD risk, not 'LDL is fine, no statin needed.'

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7. ASA (secondary prevention)

WHAT YOU SEE

A shield in the CVD ARMORY cart marked ASA with a green check on 2nd PREVENTION and a question mark on 1st PREVENTION — the shield = aspirin, checked for secondary but not routine primary prevention.

WHAT IT ENCODES

Aspirin (75–162 mg/day) is indicated for SECONDARY prevention in diabetics with established ASCVD. For PRIMARY prevention it is NOT routine — bleeding risk largely offsets benefit (ASCEND trial); reserve it for selected higher-risk patients after a bleed-vs-benefit discussion. Mnemonic: 'ASA after the event, not before.'

BOARD TRAP

Routine aspirin for PRIMARY prevention in diabetics is the WRONG answer — it is no longer recommended for most. Discriminator: give aspirin to a diabetic with prior MI/stroke/PAD (secondary prevention); do not start it reflexively in a low-/moderate-risk diabetic with no ASCVD.

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8. Beta blocker / BP

WHAT YOU SEE

A war hammer in the armory labeled BETA BLOCKER — the heavy weapon stands for the post-MI/HFrEF beta-blocker, a core BP/cardioprotective tool.

WHAT IT ENCODES

Beta-blockers are indicated post-MI and in HFrEF for diabetics (mortality benefit outweighs metabolic concerns). Aggressive BP control (goal generally <130/80) is one of the highest-yield interventions to cut macrovascular events; ACE inhibitors/ARBs are preferred first-line, especially with albuminuria. Mnemonic: 'BP control beats glucose control for the heart.'

BOARD TRAP

WRONG to withhold a beta-blocker from a diabetic post-MI out of fear of masking hypoglycemia — the cardioprotective benefit clearly outweighs this, and cardioselective agents are preferred. Discriminator: only nonselective beta-blockers meaningfully blunt hypoglycemia awareness; do not deny a post-MI diabetic a guideline-indicated beta-blocker.

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9. Dermopathy

WHAT YOU SEE

Mural panel labeled DERMOPATHY (shin spots)

WHAT IT ENCODES

Diabetic dermopathy = the MOST COMMON cutaneous finding in diabetes: well-demarcated brown, atrophic, scar-like macules ('shin spots') over the pretibial area, reflecting microangiopathy. It is benign and requires no treatment, but signals microvascular disease elsewhere. Mnemonic: 'shin spots = the commonest diabetic skin sign.'

BOARD TRAP

WRONG to mistake benign diabetic dermopathy for cellulitis or stasis dermatitis and treat with antibiotics — the lesions are asymptomatic, atrophic, and non-progressive. Discriminator: dermopathy is brown, atrophic, and painless on the shins, whereas necrobiosis lipoidica is a yellow-brown, shiny, telangiectatic plaque that can ulcerate.

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10. Bullosis diabeticorum

WHAT YOU SEE

Panel labeled BULLOUS DIABETICORUM (blisters)

WHAT IT ENCODES

Bullosis diabeticorum = spontaneous, tense, non-inflammatory bullae arising on acral skin (hands/feet) of long-standing diabetics, with no preceding trauma. They are sterile, heal spontaneously without scarring over weeks, and need only protective care. Mnemonic: 'blisters from nowhere that heal by themselves.'

BOARD TRAP

WRONG to assume a tense blister on a diabetic's foot is an infection or burn requiring antibiotics/debridement — bullosis diabeticorum is sterile and self-limited. Discriminator: spontaneous, painless, non-inflammatory bullae without surrounding erythema favor bullosis diabeticorum over bullous cellulitis or an immunobullous disorder.

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11. Necrobiosis lipoidica (NLD)

WHAT YOU SEE

Panel labeled NLD, yellow-brown shin plaque

WHAT IT ENCODES

Necrobiosis lipoidica (diabeticorum) = yellow-brown, shiny, atrophic plaques with telangiectasias and a violaceous border, classically on the SHINS (pretibial); the center can ulcerate. It is uncommon but highly associated with diabetes. Treatment is difficult — topical/intralesional steroids to the active border. Mnemonic: 'yellow shiny shin plaque = necrobiosis.'

BOARD TRAP

WRONG to confuse necrobiosis lipoidica with diabetic dermopathy — NLD is a larger yellow, shiny, ulcer-prone plaque, whereas dermopathy is small brown atrophic spots. Discriminator: telangiectasias visible through a shiny atrophic center and risk of ulceration point to NLD, not the benign, never-ulcerating shin spots of dermopathy.

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12. Acanthosis nigricans

WHAT YOU SEE

Neck panel labeled ACANTHOSIS, INSULIN RESISTANCE MARKER

WHAT IT ENCODES

Acanthosis nigricans = velvety, hyperpigmented thickening of skin folds (posterior neck, axillae, groin), a clinical marker of INSULIN RESISTANCE/hyperinsulinemia (high insulin stimulates keratinocyte/fibroblast growth via IGF receptors). Common in T2DM, obesity, and PCOS. Mnemonic: 'velvety neck = too much insulin.'

BOARD TRAP

WRONG to dismiss new acanthosis nigricans as merely cosmetic without context — sudden, extensive onset in a thin, older adult can be a PARANEOPLASTIC sign (often gastric adenocarcinoma). Discriminator: gradual acanthosis in an obese patient signals insulin resistance, but rapid widespread onset warrants a malignancy workup.

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13. Vitiligo

WHAT YOU SEE

Hands panel labeled VITILIGO, 1DM AUTOIMMUNE

WHAT IT ENCODES

Vitiligo = autoimmune destruction of melanocytes producing well-demarcated depigmented (chalk-white) macules; it CLUSTERS with type 1 diabetes and other autoimmune conditions (Hashimoto/Graves thyroid disease, pernicious anemia, Addison). Its presence should prompt screening for associated autoimmune endocrinopathies. Mnemonic: 'white patches travel in autoimmune packs.'

BOARD TRAP

WRONG to associate vitiligo with T2DM/insulin resistance — vitiligo links to T1DM (autoimmune), whereas acanthosis nigricans links to T2DM/insulin resistance. Discriminator: depigmented autoimmune patches → check thyroid and B12; velvety hyperpigmentation → check for insulin resistance/T2DM.

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14. Scleredema

WHAT YOU SEE

Back panel labeled SCLEREDEMA

WHAT IT ENCODES

Scleredema diabeticorum = painless, symmetric, woody INDURATION and thickening of the skin over the upper back, posterior neck, and shoulders, from dermal collagen and mucin deposition in long-standing, often poorly controlled T2DM. It is chronic; glycemic control is the mainstay. Mnemonic: 'thick wooden upper back in long-standing diabetes.'

BOARD TRAP

WRONG to label upper-back skin thickening in a diabetic as scleroderma (systemic sclerosis) — scleredema lacks Raynaud, sclerodactyly, nailfold changes, and autoantibodies. Discriminator: symmetric upper-back/neck induration WITHOUT digital involvement or Raynaud favors scleredema diabeticorum over systemic sclerosis.

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15. Lipohypertrophy / rotate sites

WHAT YOU SEE

Panel labeled LIPOHYPERTROPHY, ROTATE SITES

WHAT IT ENCODES

Insulin-injection lipohypertrophy = soft, rubbery fatty lumps from repeated injection into the SAME site; injecting into these areas gives ERRATIC, unpredictable insulin absorption and glycemic swings. Prevention/treatment is ROTATING injection sites and avoiding the lumps until they resolve. Mnemonic: 'same spot = lumpy fat + lousy absorption — rotate.'

BOARD TRAP

WRONG to escalate the insulin dose for unexplained glucose variability without examining the injection sites — undetected lipohypertrophy (with erratic absorption) is a common, reversible cause. Discriminator: palpate the injection sites first; rotating away from lipohypertrophic areas often restores predictable control without raising the dose.
